

Paragraph

v

SAYED MOHAMED TAN

AWY

+ pia itfusion st
Sagittal TMs

axial [LOCATION], T2and [HOSPITAL],

DINGS:

Left temporo-g6c

to-parietal cortical/subcortical are
ctions seen with restricted ai
Associated

and patches of recent

'ni fusion. No blood sign'
multiple innumerable recent
snvolving the inner border zone
'erebellar hemisphere

intensity
infarcts with rest
of the left fronto-p

ted diffusion

tal lobes and 1

right posterior parietal wedBe shaped large area of [LOCATION] like
surrounding guotic tssues an¢ exerting
ventricle.

jena
faild mass effect on the right

«periventricular sheets and contiuert patches of high [LOCATION] FLAIR
denoting small vessels [NAME]:

«prominent cerebral ventricles 'and mildly dllated cortical
spaces.

[NAME]) and extra axial CSF

© Normal size and parenchymal signal intensity pattern of the cere

lum, and
brain stem.

4s No shift of the midline structure®

© No intra or extra axial flow

© No extra-axial collections:

* Intact cranio-cervical junction

upright eye globe

VISION:

status 71/72 signal (2 O18 hemorrhage)

left hemisphere and anterior border zone (watershed areas) as well as right
cerebellar hemisphere. Recent non-hemorrhagic [NAME] likely thrombo-
embolic disease,

right posterior parietal large encephalomalacia? (old infarction)

© technical brain scans OM [NAME] vessels leukoencephalopathy

age related involutational BN changes:

: 'much obliged

Jyehata, [LOCATION]

[DATE] PM

pr. Musial

pate [NAME]

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